

[PROVIDER-NAME]
[PROVIDER-NAME]
[PROVIDER-NAME]

Our Ref: (T) DRF/JN/[MED-ID]

[DATE]

Clinic Letter : [DATE]
[PROVIDER-NAME]
[ADDRESS]
[ADDRESS]
[ADDRESS]
[POSTCODE]

Division of Unscheduled Care
[ADDRESS]
[ADDRESS]
[ADDRESS]
[ADDRESS]
[POSTCODE]

Telephone: [PHONE] Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME], [ADDRESS]. [POSTCODE]
DOB: [DATE], Hospital Number: [MED-ID], NHS Number: [NHS-NO]

I reviewed this lady today. Her colonoscopy showed a total colitis and [NAME] biopsies show m chronic inflammatory bowel disease without specific features. [NAME] appearances at endoscopy those of a Crohn's colitis where as on histology they would be consistent with ulcerative col think [NAME] best categorisation for [NAME] time being would be indeterminate colitis. She co lot of abdominal pain. It is rare for this to be a feature of inflammatory colitis without sm involvement. I suspect that her pain relates as much to her multiple sclerosis as to her coli should be treated with a pain-modifying agent such as amitriptyline.

With regard to [NAME] Crohn's disease, unfortunately [NAME] prescription she was given was in she should have been reducing her prednisolone by 5 milligrams weekly not daily. I have there her to increase to 35 milligrams daily for a week and then reduce by 5 milligrams per week th have given her my standard counselling for azathioprine and an azathioprine information leafl have checked her TPMT level. We will review her in about three weeks' time with [NAME] TPMT w aim of then starting her on azathioprine as long-term maintenance.

With kind regards

Yours sincerely

[PROVIDER-NAME]

Consultant Physician/Gastroenterologist

Honorary Senior Lecturer in Medicine

Regional Specialist Adviser for Gastroenterology

cc:

[NAME]

[ADDRESS]

[NAME]